جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : انتصار قناوي هلالي احمد
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** حلوان
رقم البطاقة : 28304072604029 **رقم التليفون :** 01242202319 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 07-04-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: انتصار قناوي هلالي احمد
National ID: 28304072604029

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



انتصار
فناوى هلالى احمد
٣ ش الجامع-عزبة زين
حلوان - القاهره

٢٩٠٤٠٧٢٦٠٤٠٢٨٣

JL5051175

٢٨٣٠٤٠٧٢٦٠٤٠٢٩

٢٠٢٢/٠٧

بدون عمل

انثى مسلمة متزوجة

محمد شعبان محمد احمد

١ البطاقة سارية حتى ٢٠٢٩/٠٧/١٤

موجهين

Patient's Name:	**Intisar Kanawi Helali**
Date:	**2026-05-17**
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS: As compared to last study dated on 2/8/2025

(I) Chest:

- **Right mastectomy with clear operative bed , no masses or collection .**
- **Right upper lobe and middle lobe subpleural reticulations likely post radiotherapy.**
- **Still noted bilateral tiny pulmonary nodules , the largest at right lower lobe anterior segment measures 5mm .**
- **Patent central air ways with well inflated both lungs.**
- **Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.**
- **No CT evidence of hilar or mediastinal lymph node enlargement.**
- **No pleural or pericardial collections seen.**
- **No gross cardiac abnormality.**

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: انتصار قناوي هلالي **MR. :** 180	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

Pt came today for ibrance 125 1*1*21 tlc 2.9/1.4 zometa 4 mg after labs

Iaowlc-Lobular Ca 82 L ER: PR: HER 2: Ki67: 718 ve Sonomammography: 10/2022 BCS + SLNB IN YS Mets Workup: T3 No Mo TNM Adj AC X 4 TaxwoLX 12 PORTIH Recommendations: zoladex + tam (S-2023-6-20251 Bone Scan 7/2025& mets. □ MDT OUTCOME & PLAN Fer IBRANCE +FEMARA+ZOLADEX FOR C6 IBRANCE 125+EMARA+ZOLADEX IBRANCE 125 ZOLADEX10.8 SC FEMARA

A CASE OF 43 YEARS OLD FEMALE PT SINCE 2020 TUBP-LOBULAR CARCINOMA RT BREAST ER8/8 PR 7/8 HER 2-VE KI6710% 10/2022 RT BCS+SLNB LN 1/5 T3N0M0 ADJUVANT AC*4+TAXOL*12 PORT ZOLADEX+TAMOXIFEN 5/2023 TILL 6/2025 BONE SCAN 7/2025 METS MDT AT 7/2025 FOR IBRANCE +FEMARA+ZOLADEX RECVIES 5 CYCLES FROM 8/2025 TILL 1/2026 THEN CT CAP 29/12/2025 PROGRESSVIE DISESAE TO BONE BONE SCAN PENDING FOR MDT

CT CAP IS PROGRESSIVE IN SCLEROTIC LESIONS BUT BONE SCAN IS STATIONARY??? CT 12/205, BONE SCAN 3/2026 FOR REMDT REGARDING CONTINUATION OF FASLODEX/IBRANCE IF POSSIBLE - mdt written for zometa only today, labs ok

pt stopped tt from 1/2026 cts 17/5/2026 comp to 8/2026 sd of osseous lesions .bs 7/3/2026 sd of oss lesions dv3,dv12 left si joint for approval of ibrance pending mdt 4/2026 continuation of tt ai zoladex ibrance for pall rth tyo left si joint for zoladex femara

4/2026 mdt : RESTARTING FASLODEX-IBRANCE / zoladex c1 d1 ibrance +fasoladex c1d 15 after two weeks

MDT --> RESTARTING FEMARA-IBRANCE zoladex ,, For Ibrance (after labs check in outpatient pharmacy) + femara , due zoladex 30/8

تحريرا في ,

التاريخ : 19-09-2026 **الوقت :**

الطبيب المعالج : ايه مدحت بدوي محمد

يعتمد ,

Patient's Name:	Intisar Kanawi Helali
Date:	2026-05-17
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

- **(III) Examined skeleton:,**
  - Still noted widespread sclerotic lesion of dorsolumbar , sternum , both iliac bone right sacral ala and left pubic bone
  - No vertebral structural collapse or wedging.

OPINION:

Rather stationary course since the last study dated 2/8/2025

Dr.Reham Khedr , MD

CT Scan
[[DOCTYPE:CT_SCAN]]

SCAN & LABS

Name : Intisar Qanawy Helaly Ahmed
Branch : Dokki
Ref.Physician Prof : Gou stav Roussey Egypt

Date : 07-03-2026 09:24
Medical No. : 328382
Ins.Company : تعاقد مستشفيات

Tc^{99}m MDPSKELETAL SCINTIGRAPHY

History

- A 42-year-old lady, with history of right breast cancer and referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

Compared to the prior bone scan dated 21-07-2025;

- Still noted enhanced tracer uptake at DV3, DV12 and left sacro-iliac region.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **Stationary course of the few active (metastatic) osseous lesions as described.**
- **The rest of skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

السكر المصرية صحة مصر

04/27/2026

MDT FORM

Name:	Antisar Kenawi Helaly .	**MRN:**	180
Age:	43 Y	**ID:**	28304072604029
Menopausal Sta		**Phone:**	01142202319
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	REMDT

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

Sonomammography:

Mets Workup:

History:

A CASE OF 43 YEARS OLD FEMAE PT SINCE 2020
10/2022 RT BCS+SLNB LN 1/5 T3N0M0
ADJUVANT AC*4+TAXOL*12
PORT
ZOLADEX+TAMOXIFEN 5/2023 TILL 6/2025
BONE SCAN 7/2025 METS
MDT AT 7/2025 FOR IBRANCE +FEMARA+ZOLADEX
RECVIES 5 CYCLES FROM 8/2025 TILL 1/2026

THEN CT CAP 29/12/2025 PROGRESSVIE COURSE OF BONE DISEASE (SCLEROTIC BONY LESON)
BONE SCAN 7/3/2026 = STATIONARY DISEASE OF FEW BONY LESIONS

LAST MDT WAS CONDUCTED BEFORE THE BONE SCAN RESULTS AND CONSEQUENTLY REGARDED AS PROGRESSIVE AND PLANNED FOR TXT X3 CYCLES
BUT NOW BONE SCAN IS STATIONARY AND IS DATED 3/2026

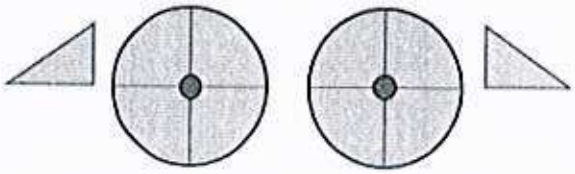
Recommendations:

FOR POSSIBILTY OF RESTARTING FASLODEX-IBRANCE TTT AS IT WAS EVIDENTLY EFFECTIVE IN DISEASE CONTROL

MDT Outcome & Plan

RESTARTING FASLODEX-IBRANCE zoladex

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ [illegible]	د/ [illegible]				

[redacted]

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / انتصار فتاوى هلالى احمد	Request No.	: 13007533
Patient No.	: 28304072604029	Request Date	: 02/10/2022
Path No.	: 2939/22	Verification Date	: 10/10/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 39 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

CLINICAL DATA: Right breast cancer, up frony surgery.

NATURE OF SPECIMEN: Right Simple mastectomy specimen & Right SLNs biopsy.

GROSS EXAMINATION:
-1- Procedure: Right simple mastectomy specimen measured 24x14cm with covering skin ellipse measured 22x12 cm .
• Tumor extent : Cut section revealed an ill defined firm greyish white area at upper outer quadrant.
• Size: measured 7x6 cm T3
• Skin involvement: unremarkable grossly
• Distance from margins: 0.5cm away from deep margin,1.5 cm from skin
2- SLNs biopsy: Fibrofatty tissue piece measured 2.5x2.5 cm enclosing 5 SLNs up to 1x1 cm
.
• Frozen section: positive SLNs (1/5) with no extracapsular extension.

MICROSCOPIC EXAMINATION:
• Histological type: Tubo- lobular carcinoma
• Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1).
• Lymphovascular emboli: positive.
• Perineural invasion: No.
• Duct carcinoma insitu: No.
• Skin involvement: Free nipple and areola .
• Lymph node involvement: Positive SLNs (1/5) with focal early occasional extra capsular extension.
• Resection Margins: Free deep margin.

DIAGNOSIS:

Verified by :
DR.Menna Nabil

Room No. (6) - First Floor Building No. (97) - 9th Corniche - Nile St Ondonia - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Entesar Kenawy Helaly Ahmed	Date : 08/08/2022
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 1543	Pat. ID: 16505

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

- Right breast palpable mass

Report:

- ACR B: mixed fibroglandular parenchyma.
- Right Breast:
  - UOQ peripheral irregular dense mass lesion with related parenchymal distortion . No overlying microcalcification. Corresponds by U/S to (11 o'clock) irregular heterogenous hypocchoic mass lesion measuring about 28 x 14 mm. It reached 3 mm form the skin.No evidence of pectoral muscle invasion.
  - Similar smaller mass is seen at (11 o'clock) measures 12 x 11 mm (noted at 2 cm distance from the forementioned mass)
  - No pathological right axillary LNs.
  - No supra or infraclavicular lymph node.
- Left Breast:
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Right breast UOQ highly suspicious mass lesions, for histopathological assessment (BIRADS 4c)
- Normal sonomammography of the left breast (BIRADS 1)

Signature Dr. Peter Nashat

08/08/2022 4:54 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name: Entesar Kenawy Helal

Date: 13/09/2022

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: History of previous right side intervention for lump (no available pathology, now presented with pathologically proved RT IDC

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: B

RIGHT BREAST:

- **Low energy (Non contrast) images** : UOQ asymmetrical density.
- **Recombined (Post contrast) images**: UOQ middle & posterior one thirds multiple confluent variable sized heterogeneously enhancing masses, globally measured 7.5x4x5cm located at about 3 cm from the nipple .
- Normal skin / nipple / areolar complex.
- **Complementary US**: Non specific axillay lymphnodes
- **LEFT BREAST:**
- **Low energy (Non contrast) images** show no masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images:** apart from faint enhancing **(glandular)** subcentimetric nodule located inner central middle third at about 8cm from nipple with no corresponding underlying abnormality seen by complementary US , no other pathological uptake detected .
- Normal skin / nipple / areolar complex.

IMPRESSION :

Right BREAST: UOQ multifocal malignant process as described (BIRADS 6= pathologically proved)

Left BREAST: benign central middle third enhancing(**glandular**) nodule (BIRADS 2= benign)

Best regards,

Dr.Marwa Ramzy

Dr. Hanaa Abo El Maged ,MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Entesar Kenawy	Date : 11/09/2022
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Findings:

I Chest:-

- A single right upper lobe posterior segment nodule is seen measuring 6 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is seen insitu.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Opinion:

- **Single right pulmonary nodule for close follow up.**
- **No CT evidence of metastatic hepatic or osseous disease.**

Dr. ALAMIN HAIDER
Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
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Radiology Department

وزارة الصـــــحة والسكـان
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مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Entesar Kenawy Helaly	Date : 20/04/2024
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 3581/2022	Pat. ID: 16505

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.

Free right operative bed with no evidence of recurrent mass lesions
Free right infra and supra clavicular regions.

MRM with free operative bed with no evidence of recurrent mass lesions
reast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada
20/04/2024 3:06 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

NILE
SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Intisar Qanawy Helaly Ahmed
Branch : Maadi
Ref.Physician Prof : Gostaf Rosch Hospital

Date : 21-07-2025 03:45
Medical No. : 157072
Ins.Company : التامين الصحي

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- A 42-year-old lady, with history of breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Enhanced tracer uptake is noted at DV3, DV12 as well as left sacroiliac region.
- No other abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Worrisome osseous lesions at the aforementioned sites, likely metastatic.

Much obliged
Dr. Mohammed AtaAllah, MSc
Nuclear Medicine

Dr. Alaa Nagi, MD
Consultant of nuclear medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Entesar Kenawy Helaly	Date : 28/04/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 42 Year(s)
Medical ID: 3581/2021	Pat. ID: 16505

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Average size, homogenous.*

- **Kidneys: -**
  - *Both are average in size, shape and echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

***Para-aortic and pancreatic area**: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

***Uterus and adnexal** ; no abnormalities could be detected, IUCD is in place*

Opinion:

Unremarkable study

Signature Dr . Nermien Ramadan

28/04/2025 2:55 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Patient Name: Entesar Kenawy Helaly	Date : 20/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 3581/2022	Pat. ID: 16505

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: 74 years old female underwent right MRM & ALAND, followed by hormonal therapy for two years, routine follow up.

Image:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

Left breast (BIRADS 1): normal study.

Clear right mastectomy bed and axilla.

Recommendation:-

- *Annual follow up.*

Best Regards,
DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
PM 12:54 2025/07/20

Mammogram
[[DOCTYPE:MAMMOGRAM]]